

Patient 4**Report A**

6494904 12/05/2016 CT head

Clinical information: 24 hour post-thrombolysis CT head. Ongoing Lhemiplegia

Findings

Comparison made with the CT head and carotid/cerebral angiogram dated 11/05/2016. Evolution of the large right sided MCA infarct is seen with hypoattenuation involving the insula, basal ganglia and Cortex at the ganglionic level with associated Sulcal effacement.

No distinct evidence of haemorrhagic transformation is seen on this study.

Thrombus is again noted within the right M1/M2 MCA. The rest of the appearances are unchanged revealing diffuse bilateral white matter hypoattenuating areas most in keeping with microangiopathic ischaemic changes.

Report B

For Thrombolysis? Yes

Report:

Time 1st Called:

Time plain CT available on PACS:

Time of verbal report to stroke registrar: 22:50

Indication: Left-sided weakness and neglect

CT HEAD

Haemorrhage: No

Intravascular thrombus: Yes; Distal M1 and proximal M2 branches of right MCA

Acute Infarct: Yes, right basal ganglia territory and right insula. Subtle sulcal effacement also noted in the right temporal lobe and frontal temporal opercular regions

Other findings and comments: Multiple bilateral chronic lacunar infarcts in the supratentorial brain including both basal ganglia regions. Moderate chronic microvascular ischaemic change

CT ANGIOGRAM

Window settings: (intraluminal HU=220; W=440; L=165)

(ICA Stenoses NASCET method, measurements on axial 1mm sections Aortic arch and subclavian arteries: Mild mixed plaque extending into origins of the great vessels

RCCA/ICA: Mild nonstenotic atheroma the common carotid; occluded right internal carotid from origin with some patchy filling of the cavernous and terminal segments.

LCCA/ICA. Approximately 40% stenosis in the left carotid bulb with mild atheroma in the common and internal carotid vessel

RVA. Focal calcification at origin

L VA. Mild stenosis at origin Intracranial Arteries: Occluded segment of distal M1 and proximal M2 branches of the right middle cerebral artery. Paucity of flow in the distal branches. No aneurysms.

Other findings and comments: No significant abnormalities seen in the soft tissues of the neck. minimal centrilobular emphysematous change in lung apices with calcified pleural plaques. Conclusion: Acute right MCA territory infarction secondary to thrombotic occlusion of segments of vessel as described; occluded right internal carotid artery.